

Pre-consultation Summary

Perimenopause & HRT workup

To: Dr. Aleatia Burger (GP, women's health)
From: Elvera van Rooyen
Date: 18 June 2026
Consultation: Virtual, 19 June 2026 (morning)

***Note on preparation:** This document was prepared by the patient (Elvera van Rooyen) in collaboration with an AI clinical research assistant — a Claude-based persona configured for midlife women's health research. All content has been reviewed and verified by the patient. It is offered as preparation for this consultation rather than as a clinical claim or diagnosis. The patient is comfortable with this AI collaboration being disclosed openly.*

1. Patient summary

44-year-old female, perimenopausal (onset noticed ~September 2025). Digital nomad, primarily based in Tofo, Mozambique, with medical care via doctors in South Africa. No locally available Western-style GP in Mozambique; online supplement / medication ordering available.

Key clinical context:

- **Diagnoses:** Temporal lobe epilepsy (TLE); generalised dystonia; rosacea.
- **Implant:** Deep brain stimulation (DBS), rechargeable battery model, placed 2006 for dystonia. Relevant for MRI safety and any strong-EMF therapies.
- **Chronic medications:** Epitec (lamotrigine) 400 mg/day, split 150 mg AM / 100 mg midday / 150 mg PM. Rivotril (clonazepam) 2 mg/day total, taken as 4 × 0.5 mg tablets each split in half, micro-dosed at roughly two-hour intervals through the waking day. No other chronic medications.
- **Diet:** Strict LCHF / ketogenic / carnivore since approximately 2018. Chosen therapeutically — with consistent subjective benefit for seizure control, dystonia symptoms, and rosacea. Carbohydrate reliably worsens TLE symptoms. Diet is not negotiable.
- **Contraception:** Copper IUD since 2015 (replaced 2021). Previous Mirena IUD before that — notable for having coincided with worsened catamenial TLE symptoms; switching to copper IUD resolved this.
- **Anthropometry:** Height 165 cm. Weight approximately 55.5 kg (BMI ~20.4). Recent gain estimated at ~5 kg over 6 months.
- **HRT:** Not currently on HRT. Open to considering it.
- **Smoking / nicotine:** Stopped vaping ~6 months ago; was on NRT until ~4 weeks ago, now nicotine-free. Began smoking historically because nicotine subjectively reduced dystonia symptoms.

2. Reason for consultation

I would like to discuss perimenopause management — specifically the case for starting HRT, the appropriate formulation and route given my neurological history (TLE on lamotrigine), and how to coordinate the plan with my SA-based neurologist. Two symptoms I particularly want to talk about are **weight / body-composition change** and **mood (irritability, easy anger, anxiety)**.

3. Clinical picture I've been working with

3.1 The underlying hormonal pattern

In perimenopause, progesterone typically falls first and falls hard, often well ahead of estrogen, which itself does not gracefully decline but surges and crashes erratically. The widened estrogen-to-progesterone ratio ('unopposed estrogen relative to progesterone') is the dominant hormonal pattern of perimenopause and appears to account for most of my current symptom picture.

3.2 Weight and body-composition change

Approximately 5 kg of new weight gain over six months, despite no change in diet, exercise, alcohol, or lifestyle. Distribution by self-observation (no scale or tape measure available in Mozambique, so the assessment is fit-of-clothing-based) is predominantly **hips, buttocks, and lateral 'love handle' region**, with a smaller abdominal component. There has been a noticeable acceleration in the last ~4 weeks — I now need a larger clothing size.

Clinically, this is a more *gynoid* (lower-body, estrogen-pattern) redistribution than the classic late-peri/post-menopausal *visceral* shift. This is consistent with being in an earlier phase of perimenopause where estrogen is still erratically elevated while progesterone has already fallen. The pattern is therefore likely hormonally driven rather than a dietary failure — further dietary restriction would be the wrong intervention. I would value your view on what to investigate and whether HRT-based intervention would be expected to influence this trajectory.

3.3 Mood — irritability, anger, anxiety

Increased irritability and quickness to anger; episodic anxiety; 2–4 AM wake-ups with palpitations and racing thoughts; daytime variability rated up to 7/10 at worst. The biological substrate I've been thinking about is progesterone-derived **allopregnanolone** withdrawal — as progesterone falls, endogenous GABA-A tone falls with it, and the calming, anxiolytic, sleep-promoting effects with it. Clonazepam is partially compensating, but the underlying GABA system is being destabilised from above.

Alcohol produces a disproportionate emotional reaction (crying/rage) that is biochemically explicable as acute GABA enhancement followed by rebound glutamate excitation, stacked on a peri-destabilised GABA system, plus the pharmacology of lamotrigine (anti-glutamatergic) and clonazepam (chronic GABA-A agonist). I have reduced alcohol intake accordingly.

3.4 Hormone-sensitive TLE — important specific factor

Historically my TLE episodes clustered around menstruation (catamenial pattern), and were specifically **worse** on the Mirena IUD and resolved on the copper IUD. This empirically confirms that the temporal lobe in my case is hormone-responsive. The perimenopausal shift in the estrogen-to-progesterone ratio is therefore biologically the wrong direction for seizure threshold — estrogen is proepileptic, progesterone (via allopregnanolone) is antiepileptic. This adds a neurological rationale for replacing progesterone, not only a symptomatic one. Bioidentical micronised progesterone is metabolised to allopregnanolone; **synthetic progestins (e.g. levonorgestrel in Mirena, MPA, norethisterone) are not**, which likely explains why Mirena worsened my TLE.

3.5 Family history

- **Osteoporosis:** strong maternal-line cluster (mother, all maternal aunts, grandmother, great-aunts). Combined with my low BMI (~20.4), historically low exercise, and imminent estrogen decline, my fracture-risk trajectory is concerning. A baseline DEXA scan feels important.

- **Breast cancer:** maternal-line cluster (three maternal aunts; maternal grandmother's sisters; likely maternal great-grandmother). No first-degree relatives affected. I'm aware that current evidence (2024–2025, including the November 2025 FDA labelling changes and the UK Cancer Genetics Group guidance) is that this family pattern does not meaningfully alter the HRT calculation when the formulation is transdermal estradiol + bioidentical micronised progesterone, but I'd welcome your view and would value a referral pathway for BRCA1/2 genetic counselling for completeness of personal information and to inform surveillance schedule.

3.6 Other relevant factors

- **Chronic stress 2022 → August 2025** self-rated at 9/10 (now declining). Three years of likely chronic cortisol exposure; HPA axis recovery probably still in progress, contributing to current mood, sleep, and possibly weight pattern.
- **Hydration:** approximately 750 ml of pure water/day plus several cups of coffee. On a ketogenic protocol in a tropical climate, low. May contribute to afternoon cognitive fatigue, 2 AM palpitations, occasional dizziness, and constipation tendency.
- **Exercise:** low historically, but began a structured routine (stretching, yoga, pilates, light strength training) approximately 5x/week in mid-May 2026.
- **Bleeding pattern change:** cycle duration unchanged but bleeding shortened from a consistent 7 days to 3–5 days, with the heavy phase compressed to ~1 day and the residual flow described as brown and clunky/sticky. Most likely anovulatory/oligo-ovulatory perimenopausal pattern; pre-HRT pelvic ultrasound feels appropriate.
- **Vaginal dryness** emerging.

4. HRT framework I have been considering

Subject entirely to your clinical judgement and neurology coordination, the framework I've been considering and would value your view on:

- **Transdermal estradiol** (patch or gel) — *not* oral. Oral estrogen induces UGT1A4, which metabolises lamotrigine, and can reduce lamotrigine serum levels by up to ~50%. At 400 mg/day for TLE this is a meaningful seizure-threshold risk. Transdermal estradiol largely bypasses this because it avoids first-pass hepatic metabolism. The British Menopause Society / Women's Health Concern (Jan 2024 factsheet) and Epilepsy Action both explicitly recommend transdermal over oral estrogen for women on lamotrigine.
- **Oral micronised progesterone** (Utrogestan / Prometrium) at night — *not* a synthetic progestin. Micronised progesterone is metabolised to allopregnanolone (supports GABA tone, sleep, anxiety, and seizure threshold). Synthetic progestins do not have this effect and, in my case, the Mirena (levonorgestrel) clearly worsened my TLE.
- **Maintain strict ketogenic diet** through HRT initiation — not changing two variables at once. The diet does real work on my TLE.
- **Lamotrigine monitoring:** baseline serum level before starting HRT, then a repeat at 4–6 weeks, coordinated with my neurologist.
- **Copper IUD retained** for contraception until full menopause confirmed.

5. Topics I'd like to cover in this virtual consultation

(Focused on what is achievable in a virtual first consultation — discussion, decision-making, planning. In-person tests, scans, and referrals are listed separately below.)

- Your overall view on the HRT framework above — agreement, modifications, alternatives.
- The weight / body-composition change — your interpretation of the gynoid distribution and recent acceleration, and whether HRT would be expected to influence the trajectory.

- The mood / GABAergic destabilisation picture — agreement with the framing, and your view on the alcohol interaction.
- Coordination strategy with my neurologist — what you'd like communicated to them and in what sequence.
- Your view on rosacea & estrogen for me specifically (the literature is bidirectional — some women improve on HRT, some flare).
- Sleep support during peri while we work toward HRT initiation — specifically the 2–4 AM wake pattern.
- Practical guidance on lifestyle pieces I'm already working on: hydration, electrolytes, resistance training for bone density, alcohol reduction.
- Realistic timeline for the workup and HRT initiation if green-lit.

6. Tests, scans & referrals I'd like to arrange from this consultation

These would need to be organised after the virtual consultation, via SA-based labs (Pathcare / Lancet / Ampath) or via Lancet Moçambique in Maputo where possible.

Bloodwork — proposed panel:

- **Reproductive hormones:** FSH, LH, estradiol, progesterone (timed to ~day 21 if cycle permits), total and free testosterone, SHBG, DHEA-S, prolactin, AMH.
- **Thyroid (full panel):** TSH, free T4, free T3, reverse T3, anti-TPO, anti-thyroglobulin.
- **Metabolic:** fasting insulin, HOMA-IR, fasting glucose, HbA1c, ApoB, hsCRP, full lipid panel, GGT, ALT, AST.
- **Micronutrients:** ferritin, transferrin saturation, B12, folate, 25-OH vitamin D, RBC magnesium, zinc.
- **Drug level:** baseline lamotrigine serum concentration (pre-HRT).
- **Optional:** morning cortisol (or 4-point salivary cortisol if accessible) given prior 3-year high-stress period.

Imaging & referrals:

- **Baseline DEXA scan** (lumbar spine + femoral neck) — given strong maternal-line osteoporosis history and low BMI. Important to establish trajectory now while pre-menopausal.
- **Transvaginal pelvic ultrasound** — standard pre-HRT, with added value given the changed bleeding character.
- **BRCA1/2 genetic counselling referral** — for information and to inform surveillance, independent of the HRT decision.
- **Coordination letter to neurologist** — to agree the lamotrigine monitoring plan before HRT initiation, and to flag the recent NRT cessation for dystonia symptom monitoring.
- **Mammogram** — if not already current, baseline for surveillance.

7. Key references

(A small set of the sources I've drawn on for the framing above, in case useful for cross-reference.)

- British Menopause Society / Women's Health Concern — *Epilepsy, the menopause and HRT*, January 2024 factsheet.
- Epilepsy Action — *The menopause and HRT* (UK).

- Sabers et al. — *Ethinyl estradiol, not progestogens, reduces lamotrigine serum concentrations*, Epilepsia (PubMed 16146436).
- Reimers et al. — *Hormone replacement therapy with estrogens may reduce lamotrigine serum concentrations* (PubMed 27805259).
- UK Cancer Genetics Group — *HRT and breast cancer risk in those with a family history*, January 2025 guidance.
- FDA — November 2025 labelling change removing black-box warnings on menopausal hormone therapy products.
- AACR — *Menopausal hormone therapy may not pose breast cancer risk for women with BRCA mutations*, December 2025.
- *Lancet Diabetes & Endocrinology* — *Menopausal hormone treatment and breast cancer*, 2025.

Appendix A — Patient-completed intake (Hormone Q&A)

A structured intake questionnaire was completed by me ahead of this consultation. The questions and my answers are reproduced below, lightly edited for clarity. Some items have been integrated into the main summary above; this appendix is provided so my unfiltered answers are also available to you should the detail be useful.

1. Cycle & periods — days between last 3 periods

Roughly 26–32 days.

1. Last period start date

Friday, 1 May 2026.

1. Current bleeding duration

Variable, 3 / 4 / 5 days. Previously was always 7 days.

1. Flow vs 5 years ago

Heavier for about 1 day then almost nothing — previously heavy for 2 or 3 days then tapered. The heavier fluid character has also changed; more brown and clunky / sticky.

1. Inter-menstrual spotting

No.

1. When cycles first started changing

Around mid-2025.

2. Sleep — usual bedtime

9 PM.

2. Time to fall asleep

I use deep breathing and lymph-drainage exercises before bed to stimulate the parasympathetic nervous system; functions as a short meditation, helps quiet overthinking.

2. Night wake-ups

Worse particularly when I have had alcohol and have skipped the pre-bed routine. More prevalent over the past ~3 months.

2. Hours of typical sleep

Variable — depends on whether I lie awake for 2–3 hours in the night.

2. Wake feeling rested

Sometimes not. I rely on a lot of coffee, so it is hard to assess clearly. I do notice my mind feels more drained in the afternoons than it used to.

3. Mood — what applies lately

Anxious / on edge; aggressive / quick to anger; tearful / crying spells; low / depressed at times; mood swings through the day; mood worse before period; sometimes hard to identify what I feel.

3. Severity, 1–10

Variable day to day, but call it 7 overall.

3. Mood pattern

Worse mornings and late afternoons depending on my routine and how I slept. Markedly worse with alcohol — I go into a crying rage that does not feel like me.

3. Past mental health history

Panic attacks when my dystonia first appeared and I didn't yet have a diagnosis. No history of depression or anxiety disorder, no PMDD.

4. Other physical symptoms

Vaginal dryness; urinary urgency at times; low libido; joint and muscle stiffness (currently using fascia-release / yoga work); occasional headache; breast tenderness; skin changes; some hair texture changes; 2 AM heart palpitations (anxiety-flavoured, breathing helps); a few episodes of feeling about to faint (USA trip); weight gain (predominantly hips / buttocks / love handles, some belly). Most of these vary with sleep, cycle phase, stress, food/alcohol.

5. Energy vs 5 years ago

Notably less, and recovers slower.

5. Cognitive symptoms

Word-finding difficulty; forgetting why I walked into a room; trouble concentrating; mental fatigue.

5. Exercise tolerance

Hard to compare — I have only just (mid-May 2026) started a structured routine after being largely sedentary, so no comparator.

6. Digestion — bowel movements per week

Variable; worse before and during period and when dehydrated.

6. Stool form

Variable.

6. Bloating

Variable — worse around the period, when I cheat on the diet, with alcohol, and when stressed or sleeping poorly.

7. Diet — time on carnivore

Approximately since late 2018.

7. Typical day includes

Muscle meat; organ meats; bone broth / connective tissue; cheese when available (not daily); eggs (not as frequent as I'd like); plenty of butter.

7. Exercise

Stretching, yoga, pilates, some strength training — 5×/week, alternating activities daily. Started mid-May 2026.

7. Water intake

Approximately 750 ml of pure water per day, plus the rest of my fluid intake from coffee.

7. Salt intake

Generous.

8. Copper IUD — duration

First inserted 2015; replaced 2021.

8. Bleeding change since insertion

Heavier (expected with copper).

8. Symptoms linked to the IUD

Worse period pain (expected). Notably, no more TLE episodes around my period since the copper IUD — a relief rather than a symptom.

9. Past hormonal contraception

Combined oral contraceptive (around 4 years, dates approximate), then Mirena IUD for approximately 5 years prior to switching to the copper IUD.

9. Surgeries

Deep brain stimulation (DBS) implant, placed 2006, for generalised dystonia. Rechargeable battery model.

9. Ongoing conditions

Temporal lobe epilepsy, generalised dystonia, rosacea.

9. Current medications

Epiletec (lamotrigine) 400 mg/day, split 150 / 100 / 150. Rivotril (clonazepam) 2 mg/day total, taken as 4 × 0.5 mg tablets each split in half, micro-dosed across the day.

9. Height & weight

165 cm; approximately 55–56 kg currently. Approximate gain of ~5 kg in the last 6 months (~2.5 kg in the last few months).

9. Smoking

Smoked in 20s; vaped for years afterwards; stopped vaping ~6 months ago; on nicotine replacement therapy (NRT) until ~4 weeks ago; now nicotine-free. Started initially because nicotine subjectively eased my dystonia symptoms.

9. Stress 1–10

Variable; call it 6 currently. Was 9–10 from 2022 through August 2025 (work-related).

10. Mum's age at menopause & experience

Final period at 53. She described it as not a good time but survivable. She used progesterone for her symptoms.

10. Family breast cancer in first-degree relatives

No first-degree relatives. Three of my mother's sisters have had it. Some of my maternal grandmother's sisters also; I believe my maternal great-grandmother died of it. All maternal-line. Nothing known on my father's side.

10. Family clots / stroke / heart disease before 60

No.

10. Family osteoporosis or hip fractures

Yes — all on my mother's side: my mother, her sisters, my grandmother, her sisters. Severity varied.

11. Supplements

Magnesium glycinate; ashwagandha (low dose, originally prescribed in 2022 by a holistic GP after full bloodwork and physical assessment); milk thistle; zinc; vitamin A; vitamin B1; folate / folic acid; CoQ10; 9-strain probiotic; desiccated liver tabs; L-tyrosine; fish oil (added about a month ago). Also a tablespoon of apple cider vinegar in a glass of water most mornings.

12. Recent bloodwork

No blood tests in the past 12 months. (Note: I did have full bloodwork done with the holistic GP in 2022 — I plan to retrieve those records and bring them as a baseline.)

13. Anything else

I don't feel like myself, don't act like myself, and don't always like myself because of the emotional / rage / 'strange' feelings. I'm uncomfortable in my clothes. I'm fairly sure I'm in perimenopause and I want to discuss whether HRT is appropriate for me — in a form that is safe given my epilepsy and lamotrigine.

Thank you, Dr. Burger. I'm looking forward to the consultation.

— Elvera van Rooyen